

Patient will increase B LE muscle strength with MMT grade of 4-/5 to improved functional mobility, good standing balance and to facilitate safe functional transfers.

Patient will ambulate safely with 2 wheeled walker with supervision/touching assistance for 50 ft. on level surfaces to facilitate safe ambulation within the facility.

Patient will increase R knee flexion strength to 4+/5 in order to facilitate increased participation with functional daily activities, increased performance with functional tasks and to return prior level of functional abilities.

Patient will improve right knee flexion strength to 4+/5 to enhance participation in daily activities, improve performance in functional tasks, and support return to prior level of function.

Patient will safely ambulate on level surfaces 50 feet using two-wheeled walker with Partial/Moderate Assistance with adequate toe clearance, with functional dynamic balance, with continuous steps, with safety during turns and with normal cadence

Pt. will maintain mod. A with bed to chair transfers and sit to stand transfers to maintain level of functional independence.

Patient will maintain LLE Strength to 3+/5 to maintain safely maneuver in/out of bed to maintain current level of function.

SKILLED INTERVENTIONS

Skilled Interventions: Patient skilled interventions necessary to achieve goals have included Neuro reed: Static and dynamic balance activities to facilitate improved proprioception and righting responses, with therapist analysis to facilitate appropriate challenge to meet LTG's, Ther Act: static and dynamic balance activities working to improve COG, functional capacity, and detection of limits of stability to decrease fall risk, functional transfer training bed<>w/c transfer training, with therapist analysis to simulate ADLs to achieve improved functional mobility and decreased fall risk, Ther Ex: B LE strengthening ther exs to

increase strength and functional capacity to facilitate improved functional performance with ADLs with therapist interjection as needed to achieve appropriate positioning and optimal effort to meet LTG's, Gait training with SCP and safety belt to improve functional mobility, Group Activity: Performed group activities including LE ther exs, UE reaching/throwing to challenge strength, balance, coordination, and detection of limits of stability working to facilitate improved functional performance with ADLs.

Skilled Interventions: Through the Skilled Maintenance program, Skilled Interventions are provided by Licensed PT and PTA including training but not limited to, Ther ex: bilat. strengthening and activity tolerance exercises to maintain B LE muscle strength, exertion, functional capacity, stability, ROM and flexibility; Ther Act : static and dynamic standing balance exercises to maintain good balance to decrease falls, to maintain safety in all levels with functional transfers , to maintain functional standing capacity, transfer training to maintain safe transfers, Education on pts. safety and fall precautions and Gait training with RW and safety belt to maintain functional mobility and steady gait and safe ambulation. Without the SMP with skilled training, pt. would be at risk for fall and functional decline.

Skilled therapeutic interventions provided to help the patient achieve their goals have included: therapeutic activities focused on static and dynamic balance training to enhance control of the center of gravity, improve functional capacity, and increase awareness of stability limits to reduce fall risk. Functional transfer training, including bed-to-wheelchair transfers, was performed with therapist guidance to simulate ADLs and promote safer, more independent mobility. Additionally, therapeutic exercises targeting bilateral lower extremity strengthening were implemented to enhance strength and functional capacity, thereby supporting improved ADL performance. Throughout all interventions, the therapist provided appropriate positioning, cues, and support to ensure optimal effort and progress toward therapeutic goals.

Neuromuscular Re-education: Implemented static and dynamic balance activities to improve proprioception and righting responses. Therapist provided ongoing analysis and adjustments to ensure appropriate levels of challenge to meet long-term goals (LTGs). Therapeutic Activities (Ther Act): Conducted static and dynamic balance exercises to enhance control of the center of gravity (COG), increase functional capacity, and improve the patient's ability to detect limits of stability, thereby reducing fall risk. Functional transfer training was incorporated, including bed-to-wheelchair and wheelchair-to-bed transitions, simulating ADLs to promote safe and efficient mobility. Therapeutic Exercises (Ther Ex): Performed bilateral lower extremity strengthening exercises aimed at increasing strength and overall functional capacity. Therapist provided guidance to ensure correct positioning and optimal effort, supporting progress toward LTGs and improved ADL performance. Gait Training: Conducted gait training with RW and safety belt to improve functional mobility and gait safety, with therapist oversight to ensure proper technique and reduce fall risk. Group Activity: Participated in group therapy activities including lower extremity therapeutic exercises and upper extremity

reaching/throwing tasks. These targeted strength, balance, coordination, and the patient's awareness of stability limits, contributing to enhanced functional performance in ADLs.

Therapeutic Activities (Ther Act): Facilitated static and dynamic balance exercises to improve control of the center of gravity (COG), enhance functional capacity, and increase the patient's ability to recognize their limits of stability—ultimately aiming to reduce fall risk. Included functional transfer training, such as bed-to-wheelchair and wheelchair-to-bed transitions, simulating activities of daily living (ADLs) to promote safe and effective mobility. Neuromuscular Re-education: Engaged the patient in both static and dynamic balance activities to enhance proprioception and righting reactions. The therapist continuously assessed and adjusted interventions to maintain an appropriate level of challenge aligned with long-term goals (LTGs). Gait Training: Performed gait training using his wheelchair and safety belt to enhance mobility and ensure gait safety. The therapist monitored and guided technique to promote correct movement patterns and reduce the risk of falls. Therapeutic Exercises (Ther Ex): Implemented bilateral lower extremity strengthening exercises to build muscular strength and boost overall functional abilities. The therapist provided real-time feedback to ensure proper positioning and maximize patient effort, supporting progress toward LTGs and improved performance in ADLs. Group Activity: Engaged in group therapy sessions involving lower extremity strengthening and upper extremity reaching/throwing tasks to promote muscular strength, eye-hand coordination and activity tolerance. These activities targeted improvements in strength, balance, coordination, and the patient's spatial awareness, all contributing to greater independence and performance in ADLs.

During this reporting period, direct, hands-on care focused on the delivery of skilled therapeutic interventions tailored to the patient's specific needs. The patient continues to require skilled therapy to meet individualized goals and maintain functional abilities. Neuromuscular re-education was provided to enhance both gross and fine motor coordination, as well as improve static and dynamic balance. These interventions targeted proprioception and righting reactions through carefully selected activities. Therapist expertise was essential in modifying the complexity of tasks to ensure appropriate challenge levels while maintaining safety and alignment with long-term functional goals. Therapeutic activities included balance training to support postural control, manage center of gravity, and increase awareness of stability limits—key components in minimizing fall risk. Bilateral lower extremity strengthening exercises were integrated to build muscular endurance and support independence in activities of daily living (ADLs). The therapist provided continuous guidance to ensure proper positioning, adequate exertion, and the development of efficient, task-specific movement patterns. The patient also engaged in structured group therapy sessions focused on lower extremity strengthening and upper extremity reaching and throwing exercises. These group activities were designed to improve strength, eye-hand coordination, balance, and stability awareness within a socially interactive setting. All interventions

required skilled oversight to monitor the patient's response, adapt activities as needed, and ensure safe and effective execution—further reinforcing the medical necessity of ongoing skilled therapy services.

Skilled therapeutic interventions provided to support goal progression included: Neuromuscular Re-education: Static and dynamic balance activities targeting improved proprioception and righting reactions, with therapist assessment to ensure appropriate challenge levels consistent with long-term goals. Therapeutic Activities: Balance-focused tasks to enhance control of the center of gravity, functional capacity, and awareness of limits of stability to reduce fall risk. Functional transfer training (bed <> wheelchair) was performed with therapist guidance to simulate ADLs and promote safer, more efficient functional mobility. Therapeutic Exercise: Bilateral lower-extremity strengthening exercises to improve muscular strength and overall functional capacity, with therapist cueing for optimal positioning and effort to support progress toward long-term goals. Gait Training: Ambulation training using an SCP and safety belt to improve functional mobility. Group Activity: Participation in group sessions involving lower-extremity strengthening exercises and upper-extremity reaching/throwing tasks designed to challenge strength, balance, coordination, and limits of stability, with the goal of improving functional performance during ADLs. E-stim: Electrical stimulation was applied to the lumbar region (right lower back) for pain reduction using premodulated waveform, continuous mode, at 80/150 Hz (CV) for 20 minutes.

THERAPEUTIC EXERCISES

Bilateral lower extremity therapeutic exercise focused on strengthening and activity tolerance exercises , seated, without resistance and moderate / 50% tactile cues/facilitation required due to compromised strength and functional activity tolerance to enhance muscle performance, enhance muscle strength and facilitate muscle power in order to enhance safety and control during dynamic movement, promote maximum cardiopulmonary performance during functional mobility, promote stability during ambulation and improve ability to ambulate without physical assistance.

Bilateral lower extremity therapeutic exercise focused on progressive resistive exercise, ankle circles, ankle dorsiflexion, ankle plantar flexion, ankle pumps, hip abduction, knees bent, hip adduction, hip flexion, knee extension and knee flexion , seated, 10 repetitions reps, 2 set(s), with 3 pound weights and moderate / 50% verbal instruction and moderate / 50% visual demonstration required due to compromised coordination, comprehension, functional activity tolerance, range of motion, strength and

muscle tone to enhance muscle performance, enhance muscle strength, improve muscle tone, facilitate optimal joint flexibility and optimize joint mobility in order to enhance safety and control during dynamic movement and improve balance for safe functional mobility. Added 3lbs. ankle weights on B LE to maximize functional strengthening and performance to enhance stability with sit to stand and to improve standing B/T.

Bilateral lower extremity therapeutic exercises were performed in a seated position, focusing on strengthening and improving activity tolerance. Exercises were completed without resistance, with moderate tactile cues/facilitation (approximately 50%) provided due to compromised strength and functional tolerance. The interventions aimed to enhance muscle performance and strength, facilitate muscle power, promote safety and control during dynamic movement, support optimal cardiopulmonary function during mobility, improve stability during ambulation, and reduce the need for physical assistance during walking.

Skilled physical therapy interventions included B lower extremity strengthening exercises with mild (25%) visual cues provided due to impaired attention, balance, coordination, functional activity tolerance, and strength. The goals were to maintain muscle performance and strength, maintain power, optimize joint flexibility and mobility, maintain muscle tone, and promote safer, more controlled dynamic movement. These interventions aimed to maintain ambulation with physical assistance and maintain balance for safe functional mobility.

Skilled PT interventions included seated bilateral lower extremity therapeutic exercises aimed at improving strength and activity tolerance, performed without resistance. Verbal cues were provided to guide pacing and reciprocal movement due to decreased strength and functional endurance. The interventions were designed to preserve muscle performance, maintain strength and power, support safe and controlled dynamic movement, optimize cardiopulmonary function during mobility tasks, and promote stability during transfers and ambulation.

Physical therapy interventions focused on seated bilateral lower extremity (B LE) activity tolerance and therapeutic exercises to enhance strength and endurance, performed without resistance. Gradually reduced cues were used to assist with reciprocal movements due to limited strength and endurance. These interventions aimed to improve muscle performance, build strength and power, facilitate safe and controlled dynamic movements, enhance cardiopulmonary efficiency during mobility tasks, and support stability during transfers and ambulation.

CONCURRENT TREATMENT

Patient participated in concurrent therapy with 1 other patient to successfully progress towards individual treatment plan/goals by addressing patient's decreased functional capacity, decreased static balance, fine motor control deficits, limitations in ROM, muscle tone limitations, pain and strength impairments ; skilled interventions included static sitting balance exercises with moderate 50%VC and TC to participate well with therapy and education on deep breathing exercises to enhance cardiopulmonary function.

Patient will benefit from concurrent therapy as an adjunct to individual therapy because concurrent therapy provides a therapeutic setting that facilitates generalization and carryover of skills learned in individual therapy, because concurrent therapy will provide the opportunity for peer modeling activity which supports learning new skills, to facilitate learning that's provided in peer therapeutic modes that would not occur in individual therapy and because patient's comprehension and insight of task demands will be enhanced by observing others.

Patient will benefit from concurrent therapy as an adjunct to individual therapy because concurrent therapy will provide the opportunity for peer modeling activity which supports learning new skills, because patient's comprehension and insight of task demands will be enhanced by observing others, to facilitate learning that's provided in peer therapeutic modes that would not occur in individual therapy and because patient's psycho social skills will benefit from the feedback provided from others in therapeutic setting and environment that would not occur via individual therapy.

GROUP THERAPY

Patient will benefit from group therapy as an adjunct to individual therapy because group therapy facilitates the assessment and carryover of skills learned in individual therapy, because group therapy will strengthen the patient's ability to physically adapt to environmental demands to improve safety, to facilitate learning that's provided in peer groups and to facilitate patient's ability to both receive and give feedback to support learning new skills.

Group therapy: Strengthening exercises on B LE with 3lbs. ankle weights, stretching exercises on B hamstrings with 10sec. hold for 10 reps to maintain muscle flexibility, trunk rotation to L/R to improve trunk mobility, weight shifting while sitting, leaning forward/backward to improve sitting balance and pursed lip breathing to improve cardiopulmonary function ansit to stand for 5x with min.A to improve functional mobility and rehabilitation fall training program to decrease risk for fall.

Patient treament included skilled group intervention consisting of 4 participants with LE weakness contributing to functional limitation. Treatment focused on range of motion exercises, stretching exercises and activity tolerance exercises with min. cueing for proper technique and execution to maintain muscle performance, muscle strength and joint mobility for safe functional transfers.

The patient took part in a therapeutic exercise group aimed at advancing their individual treatment plan and goals. The focus was on improving the patient's dynamic balance, functional capacity, pain management, safety awareness, and strength. Skilled services provided included range of motion exercises, stretching, and activity tolerance exercises, with minimal verbal cues and visual demonstrations to ensure proper technique and execution. These exercises were designed to maintain muscle strength and functional independence. The group consisted of four participants.

The patient joined a therapeutic exercise group designed to help progress their individual treatment plan and objectives. The group focused on addressing the patient's decreased dynamic balance, functional capacity, pain, safety awareness, and strength limitations. Skilled interventions included range of motion exercises, stretching, and activity tolerance exercises, with minimal verbal cues and visual demonstrations to ensure correct technique and execution. These exercises aimed to preserve muscle strength and functional independence. The group had a total of four participants.

Patient participated in a skilled group therapy session with three other residents, all demonstrating lower extremity weakness contributing to decreased functional mobility. Skilled intervention focused on therapeutic exercises including ROM, stretching, and activity tolerance training. Minimal verbal cues were provided to ensure correct technique and execution. Goals of the session included maintaining muscle strength and performance, preserving joint mobility, and improving safety and efficiency during functional transfers.

The patient participated in a skilled group therapy session alongside three other residents, all presenting with lower extremity weakness impacting functional mobility. Interventions focused on therapeutic exercises targeting eye-hand coordination, multidirectional reaching, lower extremity strengthening, and

activity tolerance training. Minimal verbal cues were required to ensure proper technique and execution. Session goals included maintaining muscle strength and functional performance, preserving joint mobility, and enhancing safety and efficiency during functional transfers.

Patient participated in a skilled group intervention with three other individuals presenting with lower extremity weakness contributing to functional limitations. Treatment focused on range of motion, stretching, and activity tolerance exercises, with minimal cueing provided to ensure proper technique and execution. The intervention aimed to maintain muscle performance, enhance muscle strength, and preserve joint mobility to support safe functional transfers.

MANUAL THERAPY

Skilled PT interventions focused on gentle passive stretching of the right ankle aimed at improving muscle flexibility and tone. Applied a foot drop splint on the right foot to address joint contracture.

Skilled physical therapy interventions focused on passive stretching of the right foot invertors and plantarflexors to preserve muscle flexibility and tone. The right lower extremity foot drop splint was donned to assist in managing joint contracture. It was noted that the CNA has been removing the splint after the patient is returned to bed.

Skilled PT interventions included stretching of shortened connective tissue in the right ankle to manage tone and maintain muscle flexibility. A foot drop splint was donned on the right ankle to prevent progression of joint contracture. It was noted that the CNA has been removing the right ankle splint after the patient is returned to bed.

Skilled physical therapy interventions focused on gentle passive stretching of the right foot targeting eversion and dorsiflexion, with a 15-second hold for 10 repetitions over 2 sets, aimed at improving muscle flexibility and managing tone. A foot drop splint was donned on the right foot to prevent further progression of joint contracture.

Skilled physical therapy interventions focused on gentle passive stretching of bilateral hamstrings and

gastrocnemius muscles to promote muscle lengthening and improve joint mobility. In addition, therapeutic massage techniques were applied to assist with tone management and to enhance muscle flexibility. These interventions were provided within the patient's tolerance and aimed at reducing muscle stiffness, preventing contractures, and supporting overall functional mobility.

THERAPEUTIC ACTIVITIES

Skilled interventions targeted crossing midline to promote independence in functional task performance, closed kinetic chain exercises to enhance functional skills, dynamic standing balance activities and strengthening exercises to improve overall task execution. Throwing activities were also incorporated to facilitate strength, coordination, and balance. The patient required mild assistance, including approximately 25% verbal cues and 25% physical support, due to impairments in attention, balance, coordination, functional activity tolerance, and strength.

Skilled interventions focused on transfer training across varied surfaces, including bed to wheelchair using a rolling walker (RW) with moderate assistance, wheelchair to shower chair with moderate assistance, and wheelchair to bedside commode (BSC) using RW with moderate assistance. Patient required mild verbal cues for sequencing and appropriate hand and foot placement to improve functional task performance. Transfers required approximately 50% physical assistance and 25% verbal instruction due to deficits in attention, balance, coordination, activity tolerance, safety awareness, and strength.

Skilled interventions addressed bending to improve dynamic balance, midline crossing to support independence in functional tasks, dynamic and static sitting balance activities, and strengthening exercises to enhance performance of daily activities. Incorporated throwing and catching tasks to improve strength, coordination, balance, and reduce reliance on verbal cues due to deficits in attention, balance, coordination, activity tolerance, and strength. Completed sit-to-stand training using the parallel bars (10 reps, 2 sets) with minimal assistance to maintain strength and limb stability for safe functional transfers.

Skilled physical therapy interventions targeted improved functional performance through various seated activities. These included placing objects out of reach to enhance dynamic skill execution, engaging in static and dynamic sitting balance exercises, and crossing the midline to promote independence in functional tasks. Range of motion techniques were also incorporated to support performance in daily activities. The patient completed 1 set of 1 repetition, requiring mild (approximately 25%) verbal cues

due to limitations in attention, balance, functional activity tolerance, postural control, and strength.

Skilled physical therapy interventions focused on seated exercises incorporating forward and left lateral reaching, as well as trunk flexion and lateral flexion with resistance. These activities targeted trunk and postural control and strength to maintain dynamic functional sitting balance and reduce fall risk."

Skilled physical therapy interventions focused on dynamic standing balance activities performed on a balance pad with a rolling walker and safety belt, under contact guard assistance. The patient was encouraged to maintain contact with the walker using only one hand to reduce external support, thereby enhancing balance, proprioception, and fall risk reduction.

WHEELCHAIR MOBILITY

Skilled PT interventions included wheelchair mobility training in the hallway with contact guard to standby assistance. Patient propelled herself using the left upper extremity and utilized the hallway handrail to increase propulsion speed.

Plan:

Continue skilled therapy 5x/week to address deficits in mobility, balance, and functional transfers. Progress bed mobility and transfer training toward modified independence. Advance standing balance tasks to improve safety with ADLs and reduce fall risk.

E- stim

E-stim was applied to the lumbar spine (right lower back) for pain reduction. Treatment parameters were: Premod waveform, continuous mode, frequency 80/150 Hz, and 6.6 volts CV for 20 minutes. The patient's skin was inspected before and after the intervention with no redness or irritation noted. Pain

level decreased from 7/10 pre-treatment to 4/10 post-treatment.

Reason for Skilled Physical Therapy Services:

1. Skilled PT services are required due to the patient's balance deficits, decreased dynamic balance, reduced functional capacity, strength impairments, edema, and impaired safety awareness. These deficits place the patient at increased risk for falls and limit their ability to safely and independently perform functional mobility and ADLs. Skilled therapeutic intervention is necessary to provide ongoing assessment, develop and progress an individualized treatment plan, and deliver targeted exercises, balance training, safety education, and edema management to improve functional performance, reduce fall risk, and enhance overall safety during mobility and daily activities.
2. Continued skilled physical therapy services are warranted due to the patient's ongoing impairments, including body awareness deficits, decreased dynamic balance, reduced functional capacity, pain, strength impairments, and joint mobility/integrity deficits. These limitations place the patient at increased risk for functional decline and falls, and continue to affect safety and independence in daily mobility. Skilled PT intervention remains necessary to address these impairments through targeted therapeutic exercise, neuromuscular re-education, mobility training, and pain management strategies to promote improved function and prevent further deterioration.
3. Continued physical therapy services are required to reduce pain, maximize rehabilitation potential, and promote independence with all functional mobility. Ongoing therapy will help improve dynamic balance, increase awareness of environmental hazards, enhance coordination, build functional activity tolerance, and support safer, more independent gait. It will also facilitate improvements in lower-extremity range of motion and strength, minimize fall risk, and promote safety awareness to enhance overall quality of life. These gains will support greater participation in daily functional activities, improved performance of functional tasks, reduced reliance on caregivers, decreased fall risk, and increased health literacy to optimize overall health, wellness, and function. Given the patient's documented physical impairments and related functional limitations, the absence of skilled therapeutic intervention places the patient at risk for contractures, reduced participation in functional tasks, falls, further functional decline, increased caregiver dependence, heightened pain, limited out-of-bed activity, muscle atrophy, and pressure injuries.
4. Skilled physical therapy services are medically necessary due to the patient's combination of complex physical impairments, including decreased functional capacity, reduced range of motion, diminished safety awareness, and significant deficits in both static and dynamic balance. The patient also presents with fine motor control impairments, joint mobility and integrity limitations, muscle disuse and atrophy, abnormal muscle tone, and paralysis/paresis contributing to global strength deficits. These impairments collectively compromise the patient's ability to safely and independently perform functional mobility and ADLs, placing them at increased risk for falls, further injury, progressive deconditioning, and loss of

independence. Skilled PT intervention is required to accurately assess movement deficits, provide targeted therapeutic techniques, adjust and progress interventions based on clinical reasoning, and implement evidence-based strategies to improve ROM, strength, coordination, neuromuscular control, balance, and safety awareness. Without skilled physical therapy, the patient is likely to experience further functional decline, increased caregiver burden, heightened fall risk, joint contractures, worsening muscle atrophy, and decreased participation in meaningful daily activities. Continuous skilled PT is essential to restore optimal functional mobility, enhance safety, and promote improved overall quality of life.

5. Skilled physical therapy services are medically necessary due to the patient's significant balance deficits, decreased dynamic balance, reduced functional capacity, impaired safety awareness, and strength limitations. These impairments substantially affect the patient's ability to safely and independently perform mobility tasks and ADLs, placing the patient at increased risk for falls, loss of balance, injury, and functional decline. Skilled PT is required to provide targeted interventions that address these complex deficits through evidence-based balance training, strengthening exercises, task-specific functional activities, and safety re-education. A licensed therapist is needed to analyze movement patterns, modify activities in real time to ensure safety, progress therapeutic challenges appropriately, and implement strategies to improve neuromuscular control, postural stability, and safe decision-making during mobility. Without skilled PT intervention, the patient is at high risk of continued decreased mobility, increased fall risk, reduced independence, and further deterioration in overall functional ability. Continued skilled therapy is essential to restore safer mobility, enhance strength and balance, improve functional capacity, and promote greater safety awareness in order to optimize the patient's quality of life.

OWENS

Skilled PT interventions

Direct, hands-on care during this reporting period focused on the following skilled interventions:

Stretching: Manual stretching of shortened connective tissues in both lower extremities to address ROM limitations, improve muscle flexibility, and prevent progression of joint contractures. **Therapeutic**

Exercise: Strengthening and performance-based exercises designed to improve muscular strength, enhance functional capacity, and optimize joint mobility. **Positioning:** Proper therapeutic positioning to

improve comfort, promote midline alignment, preserve skin integrity, and prevent skin breakdown.

Neuromuscular Re-education: Activities targeting improved righting reactions and postural control, including trunk mobility and core stability exercises to enhance overall neuromuscular function. **Safety**

Education: Instruction on safety techniques to reduce fall risk and improve the patient's ability to navigate functional tasks safely. **Group Therapy:** Participation in group therapy sessions to support

progression toward individualized treatment goals by addressing decreased functional capacity, reduced static balance, fine motor control deficits, ROM limitations, abnormal muscle tone, and strength

impairments. **Balance Training:** Static and dynamic sitting balance exercises were performed with moderate assistance, including approximately 50% verbal and tactile cueing, to promote greater

participation and improved postural stability. **Breathing Techniques:** Education and practice of deep-breathing exercises to enhance cardiopulmonary function and support overall endurance. These skilled

interventions were essential to address the patient's impairments and facilitate meaningful progress toward established therapy goals.

Reason for skilled PT services

Skilled physical therapy services are medically necessary due to the patient's combination of complex physical impairments, including decreased functional capacity, reduced range of motion, diminished safety awareness, and significant deficits in both static and dynamic balance. The patient also presents with fine motor control impairments, joint mobility and integrity limitations, muscle disuse and atrophy, abnormal muscle tone, and paralysis/paresis contributing to global strength deficits. These impairments collectively compromise the patient's ability to safely and independently perform functional mobility and ADLs, placing them at increased risk for falls, further injury, progressive deconditioning, and loss of independence. Skilled PT intervention is required to accurately assess movement deficits, provide targeted therapeutic techniques, adjust and progress interventions based on clinical reasoning, and implement evidence-based strategies to improve ROM, strength, coordination, neuromuscular control, balance, and safety awareness. Without skilled physical therapy, the patient is likely to experience further functional decline, increased caregiver burden, heightened fall risk, joint contractures, worsening muscle atrophy, and decreased participation in meaningful daily activities. Continuous skilled PT is

essential to restore optimal functional mobility, enhance safety, and promote improved overall quality of life.

The patient continues to demonstrate significant impairments including decreased dynamic balance, reduced functional capacity, strength deficits, impaired safety awareness, and ongoing abdominal pain. These deficits place the patient at increased risk for falls and injury and limit safe participation in functional mobility and activities of daily living. Skilled physical therapy services are required to provide clinical assessment, therapeutic exercise progression, balance training, pain management strategies, and task-specific functional training that cannot be safely or effectively performed independently. Ongoing skilled intervention is necessary to address safety awareness deficits, improve strength and balance, enhance functional endurance, and promote safe mobility to maximize independence and reduce fall risk.

Continued physical therapy services are medically necessary to maximize the patient's rehabilitation potential by improving independence with functional mobility, enhancing dynamic balance, increasing functional activity tolerance, and advancing gait independence. Ongoing skilled intervention is also required to improve lower extremity range of motion and strength, reduce fall risk, and promote safety awareness. These improvements are essential to enhance the patient's quality of life by decreasing reliance on caregiver assistance, increasing participation in functional daily activities, and enabling safer functional mobility with reduced risk of falls. Given the documented physical impairments and associated functional deficits, the absence of skilled therapeutic intervention places the patient at risk for decline in mobility, reduced participation in leisure activities, increased fall risk, further functional deterioration, greater dependence on caregivers, limited out-of-bed activity, and potential social isolation.